

NURSING WITH LOLO

Health Assessment

A clear head-to-toe sequence with expected, unexpected, and urgent findings.

Simple explanations - nursing priorities - memory cues - original practice - rationales

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Educational study aid - verify current instructor and facility requirements

What you will be able to do

Use these objectives to guide active recall before and after the lesson.

Learning objectives

- Organize a respectful, efficient head-to-toe assessment.
- Distinguish expected, unexpected, and urgent findings.
- Document and escalate meaningful assessment changes.
- Measure and validate vital signs using appropriate technique.
- Interpret trends in the context of the individual patient.
- Complete a multidimensional pain assessment and reassessment.

How to use this guide

- Read the simple overview once.
- Close the guide and explain the priority in your own words.
- Answer the practice items before opening the rationale.
- Return to the lowest-confidence section tomorrow.

The concept, made simple

Plain language first; precise nursing language follows.

Core explanations

- Start with a general survey: appearance, behavior, breathing, position, mobility, and immediate safety. If the patient appears unstable, switch to a rapid primary assessment and call for help.
- For a stable patient, use the same organized sequence each time so fewer areas are missed. Compare both sides when appropriate and connect findings rather than treating each system as separate.
- Assessment includes what the patient reports, what the nurse observes, focused measurements, and relevant records. A finding matters most when it is new, worsening, inconsistent with baseline, or linked to symptoms.
- Temperature, pulse, respirations, blood pressure, oxygen saturation, and pain data help the nurse recognize stability or change. Expected values vary with age, condition, activity, environment, and treatment.
- A single number never tells the whole story. Check the patient, compare the baseline and trend, and confirm unexpected readings before deciding what they mean.
- Pain is whatever the patient experiences and reports. A score is useful, but location, quality, timing, effect on function, and the patient's acceptable goal make the assessment meaningful.

Vocabulary move

- Name the body system or safety process.
- Connect the cue to the risk.
- State what the nurse should notice and do first.

Assess before you act

A focused assessment organizes the data that make the next action safe.

What to notice

- Verify identity, explain the assessment, provide privacy, clean hands, and ask about current concerns.
- Complete a general survey and review vital signs, pain, allergies, safety risks, and relevant history.
- Assess neurologic, head and neck, respiratory, cardiovascular, abdominal, urinary, musculoskeletal, skin, and psychosocial findings as appropriate.
- Inspect lines, drains, wounds, access sites, and safety equipment without disrupting therapy or sterile systems.
- Observe overall appearance and breathing before focusing on equipment readings.
- Use the correct device, site, patient position, and cuff or sensor size.
- Compare with baseline, prior values, symptoms, recent activity, and relevant treatment.
- Assess pain location, quality, onset, pattern, severity, function, associated symptoms, and patient goal.

Trend, do not snapshot

- Compare with baseline.
- Cluster related cues.
- Validate unexpected values.
- Escalate time-sensitive changes.

Priority actions and safety boundaries

The safest answer protects the client while staying inside role, order, and policy.

Priority actions

- Pause the routine sequence for any immediate airway, breathing, circulation, neurologic, or safety threat.
- Stay with an unstable patient and activate the facility response pathway.
- Compare a new finding with baseline and notify the appropriate clinician using a focused SBAR.
- Reassess after intervention or when the patient's condition changes.
- Treat the patient's visible instability as urgent even if a device reading seems normal.
- For a concerning machine value, assess the patient and verify equipment or technique.
- If position change causes dizziness, return the patient to a safe position and reassess.
- Escalate new severe symptoms or worsening trends using current criteria.

Safety warnings

- Do not continue a routine exam while a patient shows signs of acute deterioration.
- Do not force movement, remove protective devices, or interrupt therapy without verifying restrictions.
- Do not label a finding normal solely because it was documented earlier; compare it with the patient now.
- Do not chart a device value you know is unreliable without repeating or explaining the limitation.
- Do not use a routine schedule as a reason to delay reassessment of a deteriorating patient.
- Sedation and respiratory status require attention when evaluating response to pain medication.
- Do not delay escalation for severe dyspnea, chest pressure, cyanosis, new confusion, or signs of shock.

Memory that transfers

A memory aid is useful only when it leads back to clinical reasoning.

Memory hooks

- Survey first, systems next, safety always.
- Look, listen, feel-then link the findings.
- New, unequal, worsening, or symptomatic = follow up.
- See the patient, check the number, trace the trend.
- OPQRST builds the symptom story.
- Pain follow-up: effect, function, safety, next step.

Exam reset

- What can harm the client first?
- What data are missing?
- Is the client stable or unstable?
- Which action is least restrictive and within scope?

Clinical judgment in motion

During a routine assessment, the nurse notices a new facial droop and the patient's speech sounds different from the earlier handoff.

Notice

- New facial asymmetry
- New speech change
- Clear change from baseline

Interpret

- Possible acute neurologic emergency
- Routine assessment must stop

Respond

- Note the time and complete a rapid focused neurologic and immediate safety assessment.
- Activate the designated urgent-response pathway without delay.
- Communicate baseline, new findings, relevant timing, and response; reassess continuously.

Reflect

- A systematic assessment revealed a time-sensitive change because the nurse compared current and prior function.

Practice before the rationale

Choose the best answer using the cue-risk-action sequence.

Question 1

During a routine head-to-toe assessment, the patient develops labored breathing and altered alertness. What should the nurse do first?

- A. Stop the routine sequence and perform a rapid airway, breathing, and circulation assessment
- B. Finish the skin assessment
- C. Ask the patient to wait until documentation is complete
- D. Continue in the usual order

Question 2

Which findings require prompt neurologic escalation? Select all that apply.

- A. New facial asymmetry
- B. New one-sided weakness
- C. Sudden speech change
- D. Abrupt change in alertness
- E. A long-standing stable preference for the left hand

Answers, rationales, and printable review

Question 1: A

Acute instability overrides the routine assessment sequence.

Exam clue: Acute breathing plus mentation change means instability.

Question 2: A, B, C, D

New focal or global neurologic changes may be time-sensitive and require urgent evaluation.

Exam clue: Select findings that are sudden and new.

Five-point review

- Begin with a general survey and switch to rapid priorities when instability appears.
- Use a consistent sequence, compare sides and baseline, and connect findings.
- Document and escalate new, worsening, unequal, or symptomatic findings.
- Vital signs require accurate technique, context, and trends.
- Assess the patient before trusting or rejecting an unexpected number.

References

National Council of State Boards of Nursing: 2026 NCLEX-PN Test Plan - <https://www.ncsbn.org/publications/2026-nclex-pn-test-plan>

MedlinePlus, U.S. National Library of Medicine: Vital signs - <https://medlineplus.gov/ency/article/002341.htm>

MedlinePlus, U.S. National Library of Medicine: Electrolyte Panel - <https://medlineplus.gov/lab-tests/electrolyte-panel/>

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